

PRIVATE AND CONFIDENTIAL

Agenda

14:30 -14:45 (15m)	Welcome, housekeeping, and programme update Andrew Newman, ODI
14:45 -15:05 (20m)	Information about Unpaid Carers (Discussion) Neil Majithia and Andrew Newman, ODI
15:05 – 15:25 (20m)	Controlled vocabularies Neil Majithia, ODI
15:25 – 15:55 (30m)	Assessments and Plans (Interactive Session) Juliette Clewlow and Stephen Rice, Social Finance
15:55 -16:00 (5m)	Next Steps

Things to be aware of

Conflicts of interest – Please inform the chair / secretariat if you think there is a risk.

Anti-competitive activities – Please inform the chair / secretariat if you think there is a [risk of anti-competitive activity](#).

Intellectual Property - If you are concerned about the use of your contributions, please contact the secretariat.

Meeting Recording - We will record the meeting and using AI transcription tools.

Working together

1. Every voice matters
2. Please join in on the conversation, we don't want this to be one way
3. If you're happy to, keep your camera on
4. Please raise your hand if you want to come in, or drop your ideas in the chat
5. Respect the diverse viewpoints in the room, assume positive intent and remain open for different experiences
6. We will be using Miro to support our discussions today

News!

Receive our regular
newsletter

Sign up at:

- <http://eepurl.com/jnmjYs>

Website: <https://socialcaredata.github.io/> (start here!)

GitHub: <https://github.com/SocialCareData>

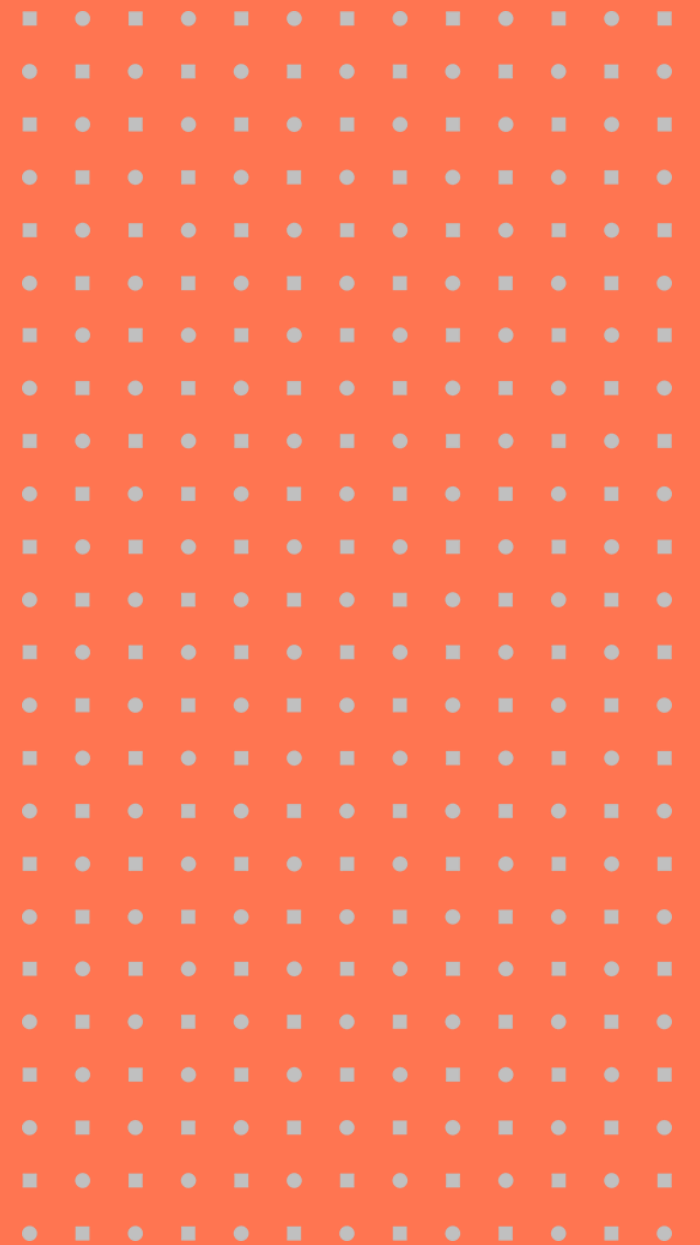
We are using GitHub to work openly:

- Store our specifications
- Track feedback on specifications (Issues)
- Openly share project documentation
- Have Discussions to inform the development of standards

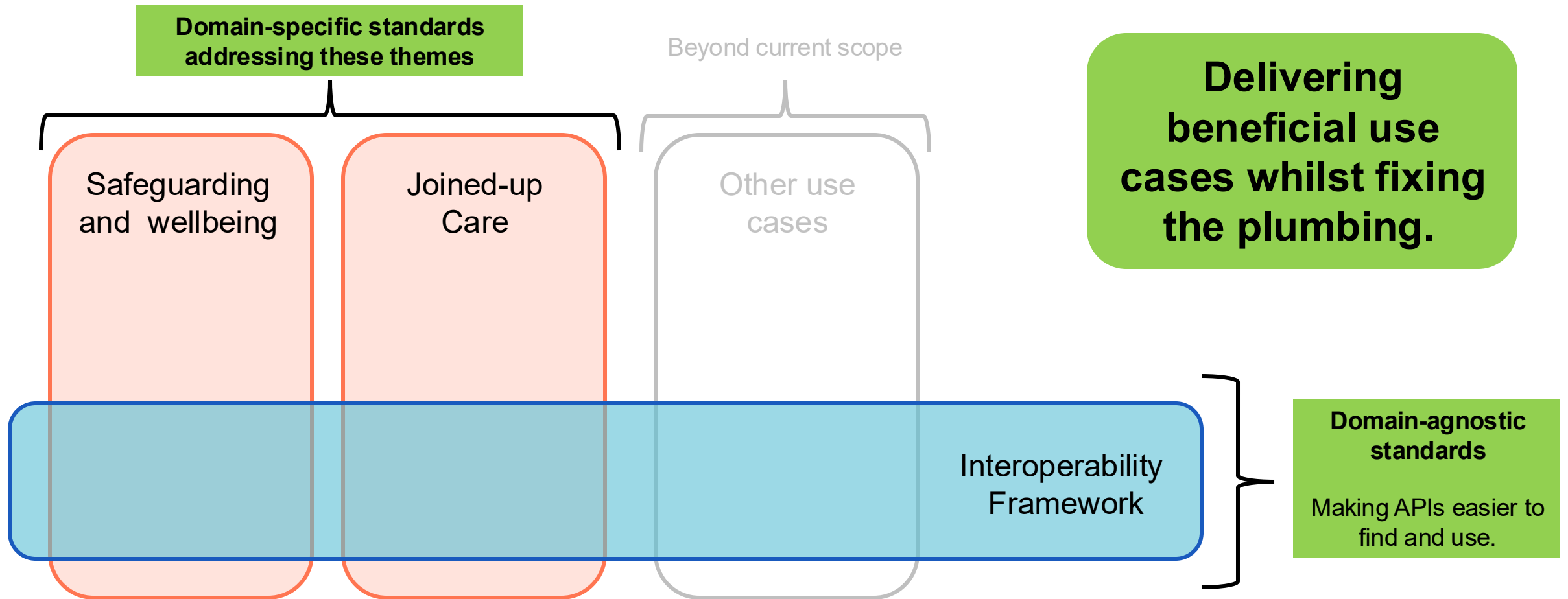
- In the future we will use GitHub to:
- Host and manage the development of any infrastructure or tools we need to develop
- Enable people to reuse our specifications and code

We will work to keep this simple and provide guidance when needed.

Programme Update

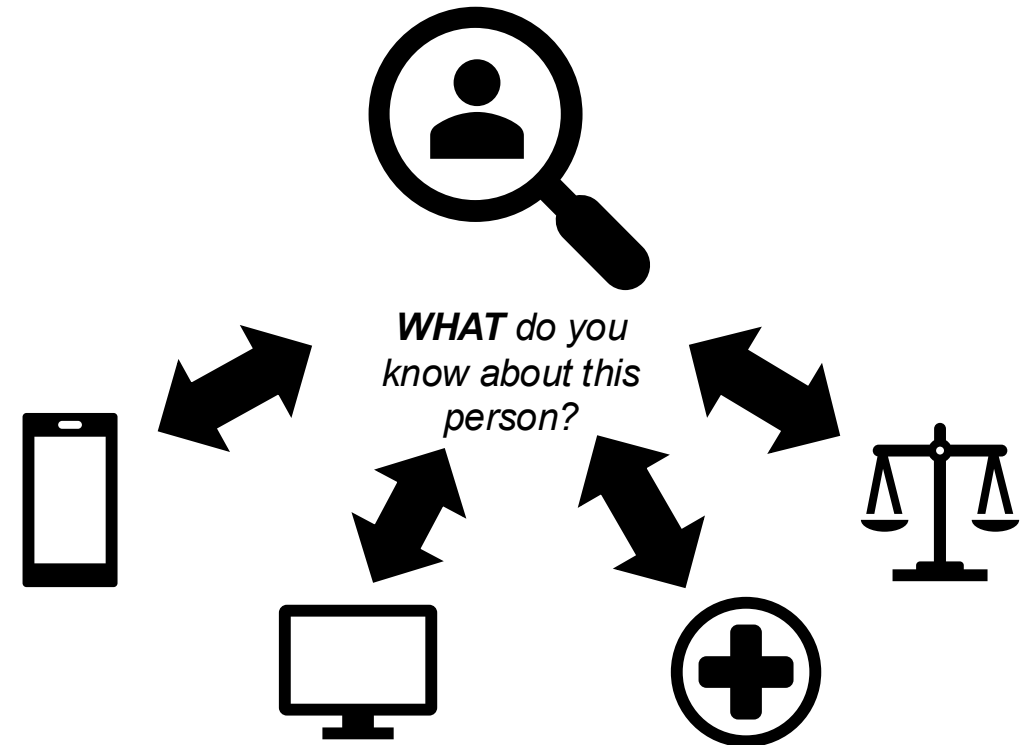
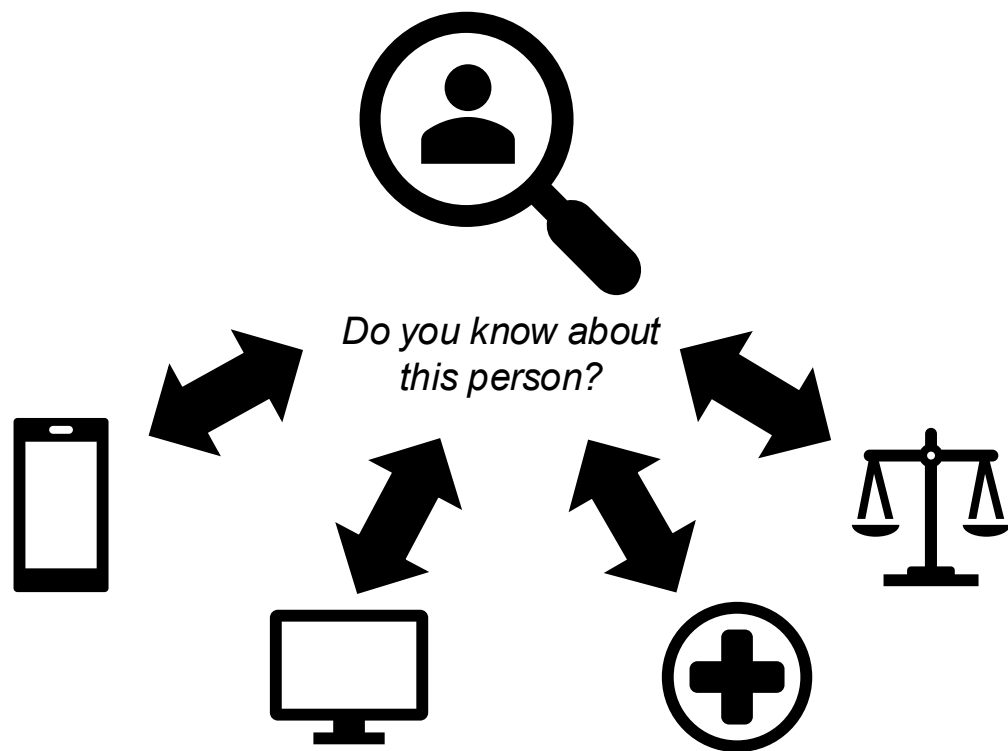


Current Standards Development Focus



Safeguarding and Wellbeing Workstream

We're focused on creating standards which allow for **more efficient and timely sharing of information between agencies to support safeguarding and wellbeing** decisions, with a focus on children's social care.



The opportunity to improve joined up care

Our priority use cases:

Multi-agency enquiries for assessment and review

- Easily accessing information to complete a **care needs assessment** e.g. *GP/mental health*
- Easily accessing information to **review a care plan** e.g. from *care providers*

Timely notifications and updates to social care

- Ensuring social workers have timely information about **hospital admissions and discharge**
- Social workers see **updates about support** provided to a person e.g. from *mental health or reablement*

Transitions from CSC to ASC

- Adult social workers can receive, track and act on information about young people **transitioning from children's to adult's services** e.g. from *CSC CMS record*

Recording and sharing assessments and plans

- **Recording assessments** easily into the CMS (incl from transcription tools)
- **Sharing care needs assessments and care plans** more easily with partners (and for analysis and reporting)

Current data model

Person standard

Services
standard

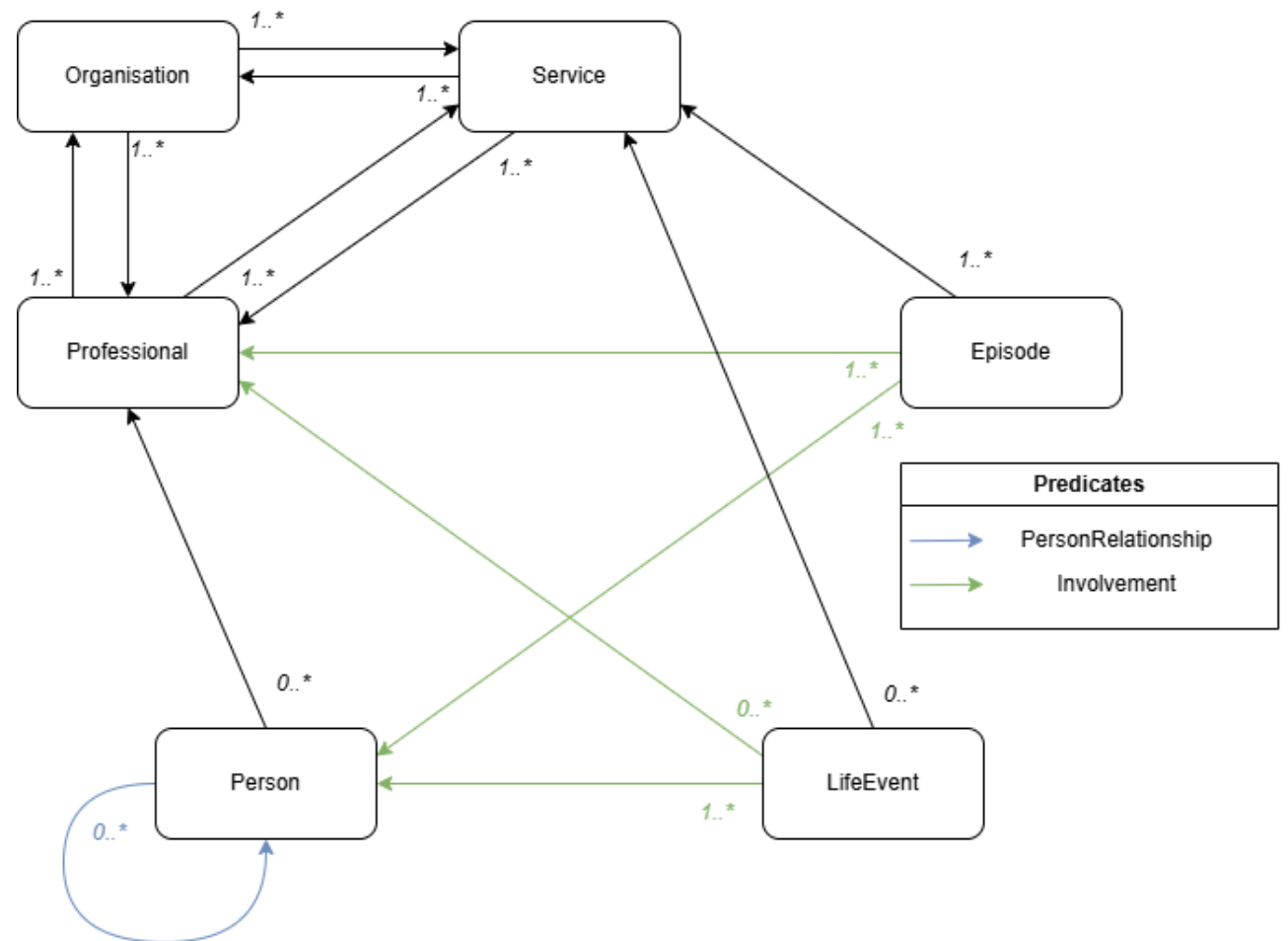
Professional
standard

Relationship
standard

Organisation
standard

Life Event standard

Service Episode standard



Interoperability framework

Reducing the cost and fragility of API integrations in and around social care by establishing shared standards for APIs.

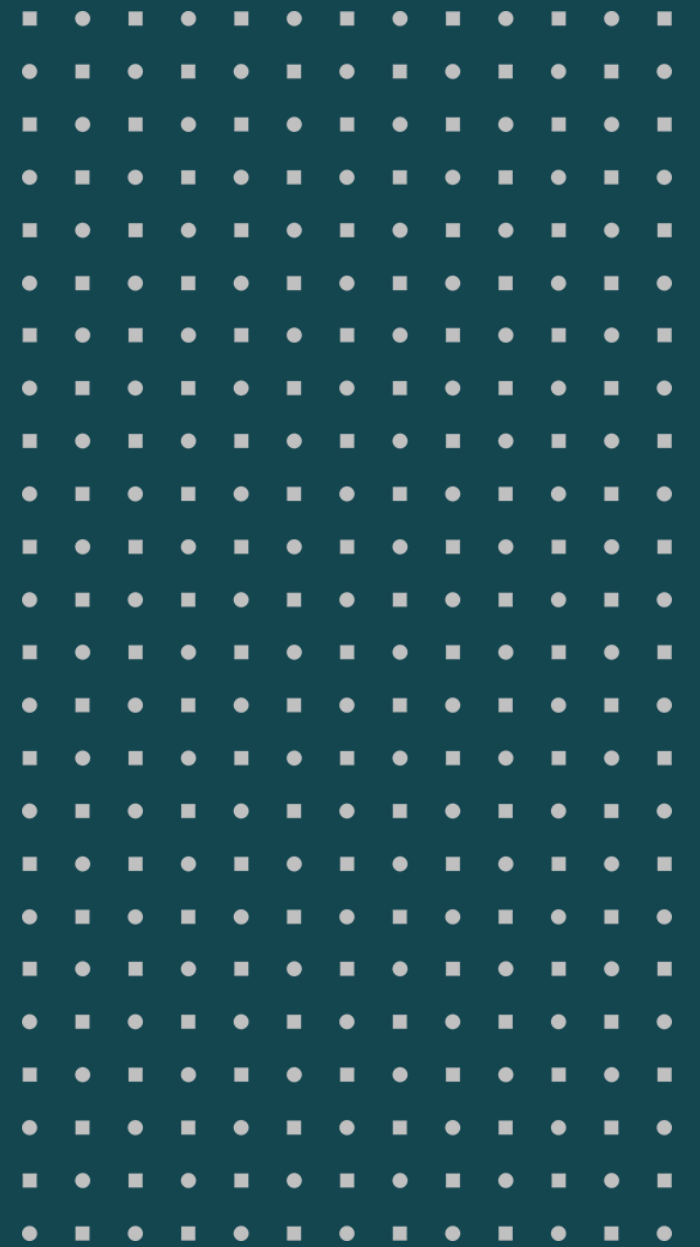
1. Discoverability - Making APIs easier to find

- **A shared API Register:** We could specify a machine-readable catalogue of all available APIs.
- **Standardised API Documentation:** We could specify standards for documenting APIs as a first-class part of the product, with clear signaling of stable vs. deprecated versions.

2. Accessibility - Making APIs easier to use

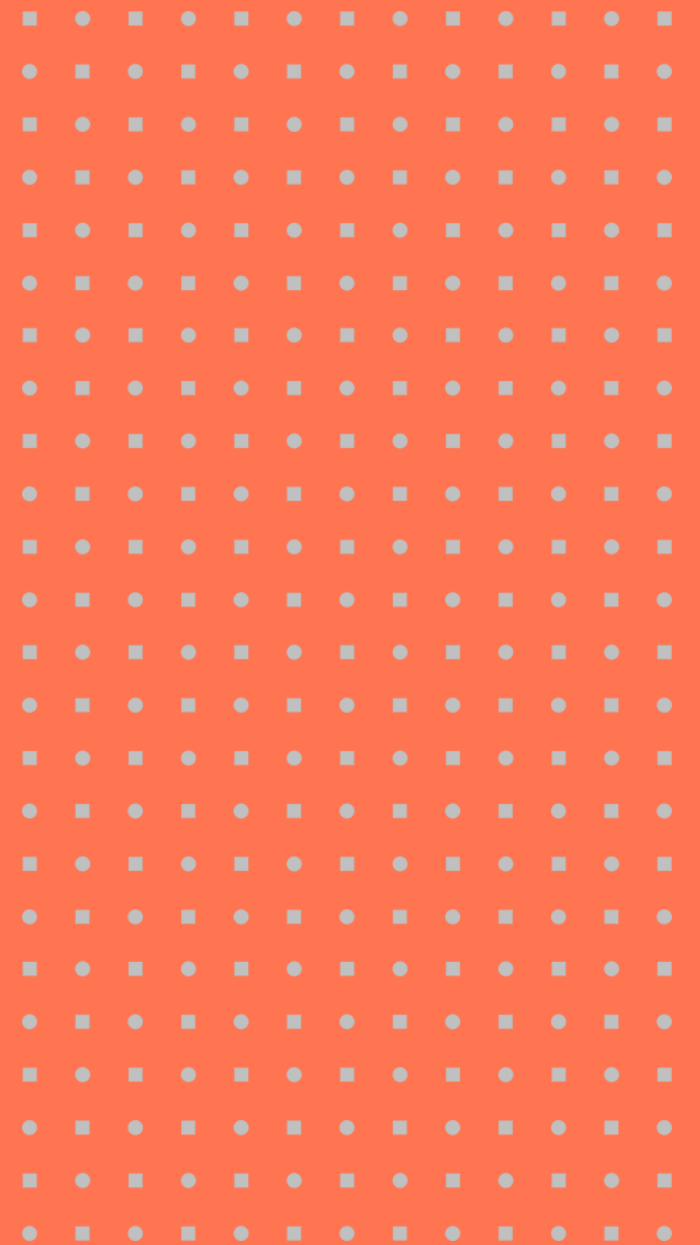
- **Structural Standards for APIs:** We could specify that APIs should conform to REST with OpenAPI specifications.
- **Identity & Access Management (IAM):** We could specify that APIs utilise OAuth 2.0 / OIDC for authentication.
- **Versioning:** We could set standards to ensure API versioning is clear and easy to understand.
- **Improving Developer Experience:** We could specify the provision of sandbox/test environments.

Any questions?

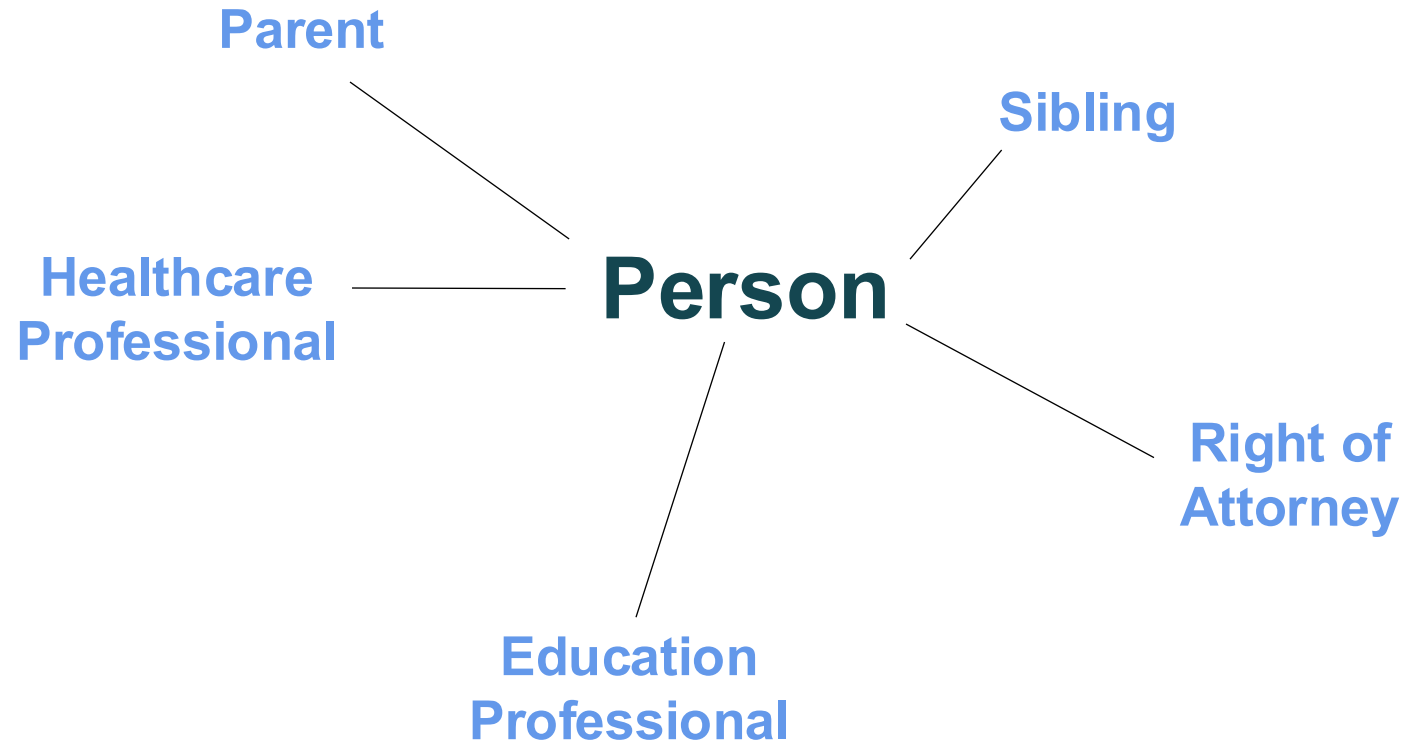


Unpaid carers

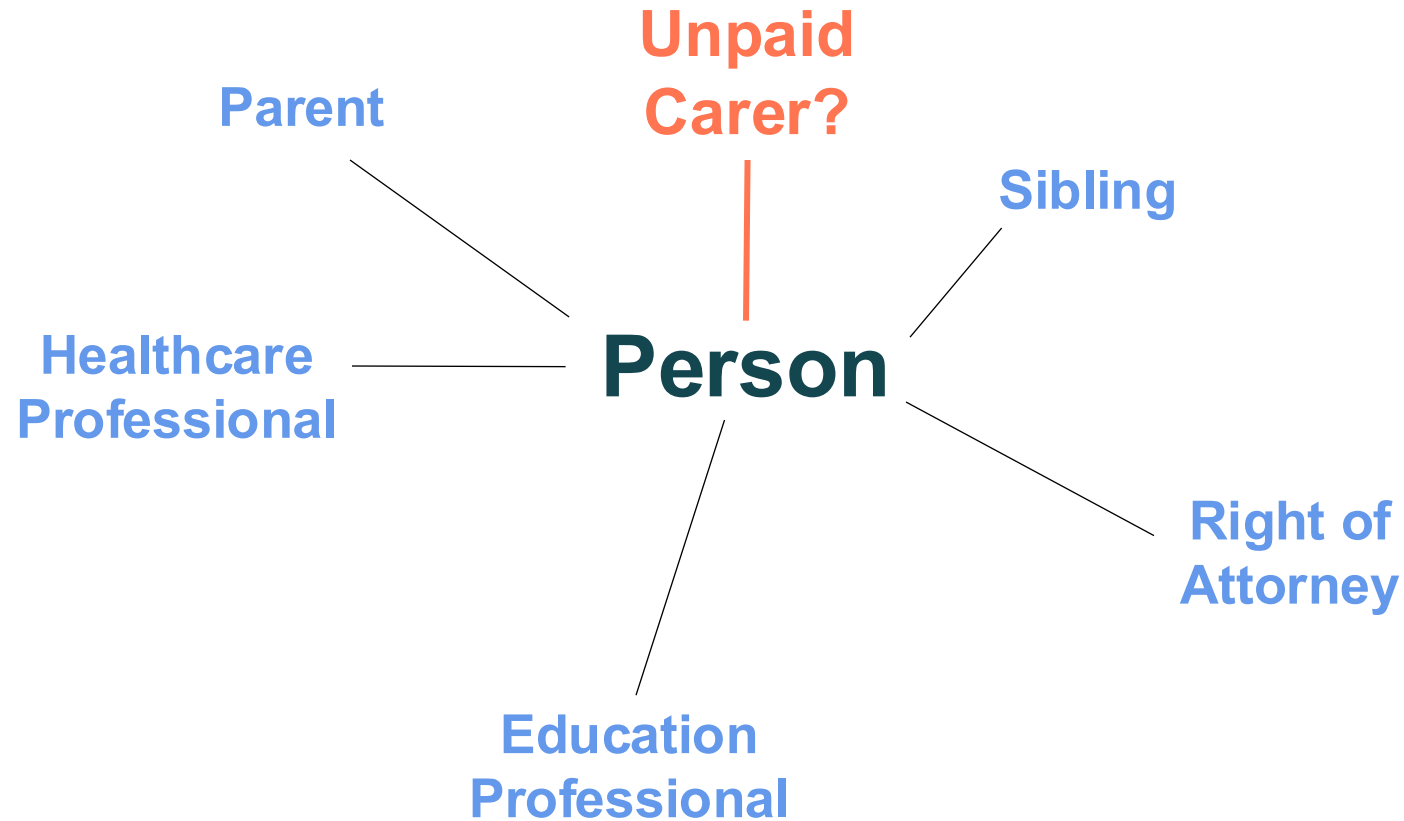
Discussion



Our data model describes the network of people around a subject of care



Our data model describes the network of people around a subject of care



Unpaid carers in Adult Social Care

An unpaid carer is a **non-professional individual** that voluntarily looks after a subject of care. A carer can often be a parent, but adult children can be unpaid carers for their parents, as can extended family or friends.

This is relevant for Children, too. A carer is generally the person with parental responsibility, but this cannot be assumed from biological relation.

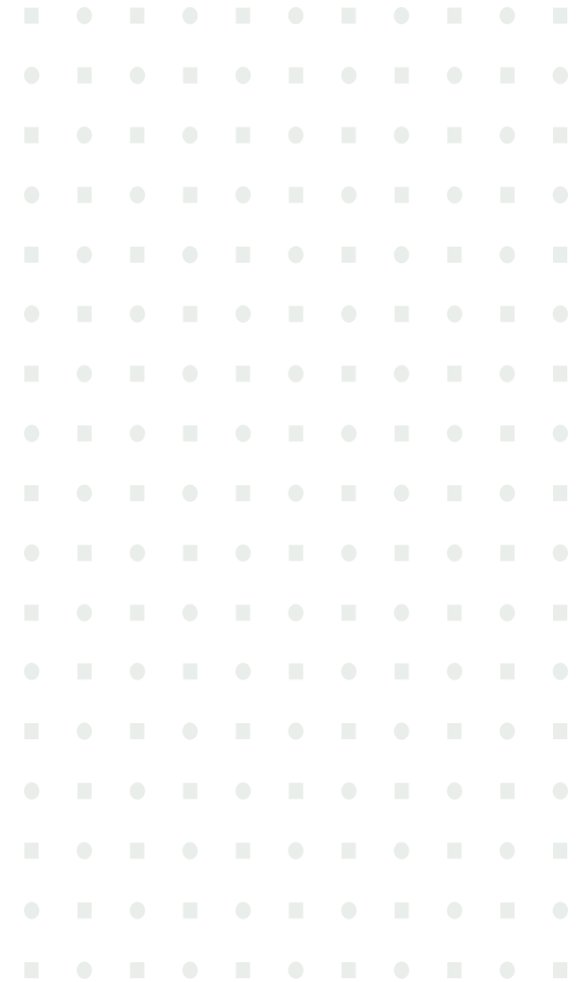
Are young carers transition assessments happening in line with the Care Act? [The Children's Society research](#) said that only 13 per cent of young adult carers reported they had received a transition assessment, with some having this happen after they became 18. Confusion among practitioners about this process was also reported.

<https://www.local.gov.uk/our-support/partners-care-and-health/cqcs-new-assurance-framework/unpaid-carers-and-care-quality>

Why do we need to describe this information?

- At the minimum, a person's unpaid carer can be the **first point of contact for professionals** to understand their health and social care circumstances.
- Unpaid carers also administer medicine, assist with mobility, etc.
- Moreover, they can be **eligible for support services or social worker interventions**
- + management of miscellaneous safeguarding risks, ASC and CSC

We understand that govt. Departments (and CQC) are increasingly looking to focus on unpaid carers and how they are represented in data, especially in CLD.



How do we currently describe unpaid carers? (1)

Subject of care: Person

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "Person",
  "Identifier": [
    {
      "value": "1234567890",
      "system": "https://fhir.nhs.uk/Id/nhs-number"
    }
  ],
  "Name": {
    "familyName": "Doe",
    "givenNames": ["John"],
    "use": "official"
  },
  "DateOfBirth": {
    "date": "1999-06-15"
  },
  "Address": [
    {
      "line1": "123 Fake Street",
      "city": "London",
      "postcode": "SW1A 1AA"
    }
  ]
}
```

How do we currently describe unpaid carers? (2)

Subject of care: Person

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "Person",
  "Identifier": [
    {
      "value": "1234567890",
      "system": "https://fhir.nhs.uk/Id/nhs-number"
    }
  ],
  "Name": {
    "familyName": "Doe",
    "givenNames": ["John"],
    "use": "official"
  },
  "DateOfBirth": {
    "date": "1999-06-15"
  },
  "Address": [
    {
      "line1": "123 Fake Street",
      "city": "London",
      "postcode": "SW1A 1AA"
    }
  ]
}
```

Someone else: ConnectedPerson

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "ConnectedPerson",
  "Name": {
    "familyName": "Doe",
    "givenNames": ["Jane"]
  },
  "PersonalContact": {
    "telephone": ["07123456789"]
  }
}
```

How do we currently describe unpaid carers? (3)

Subject of care: Person

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "Person",
  "Identifier": [
    {
      "value": "1234567890",
      "system": "https://fhir.nhs.uk/Id/nhs-number"
    }
  ],
  "Name": {
    "familyName": "Doe",
    "givenNames": ["John"],
    "use": "official"
  },
  "DateOfBirth": {
    "date": "1999-06-15"
  },
  "Address": [
    {
      "line1": "123 Fake Street",
      "city": "London",
      "postcode": "SW1A 1AA"
    }
  ]
}
```

Someone else: another Person known to services

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "Person",
  "Identifier": [
    {
      "value": "0987654321",
      "system": "https://fhir.nhs.uk/Id/nhs-number"
    }
  ],
  "Name": {
    "familyName": "Doe",
    "givenNames": ["Jane"],
    "use": "official"
  },
  "DateOfBirth": {
    "date": "1990-04-23"
  },
  "Address": [
    {
      "line1": "123 Fake Street",
      "city": "London",
      "postcode": "SW1A 1AA"
    }
  ]
}
```

How do we currently describe unpaid carers? (4)

Subject of care: Person

```
{
  "@context": "https://socialcaredata.github.io/spec/",
  "@type": "Person",
  "Identifier": [
    {
      "value": "1234567890",
      "system": "https://fhir.nhs.uk/Id/nhs-number"
    }
  ],
  "Name": {
    "familyName": "Doe",
    "givenNames": ["John"],
    "use": "official"
  },
  "DateOfBirth": {
    "date": "1999-06-15"
  },
  "Address": [
    {
      "line1": "123 Fake Street",
      "city": "London",
      "postcode": "SW1A 1AA"
    }
  ]
}
```

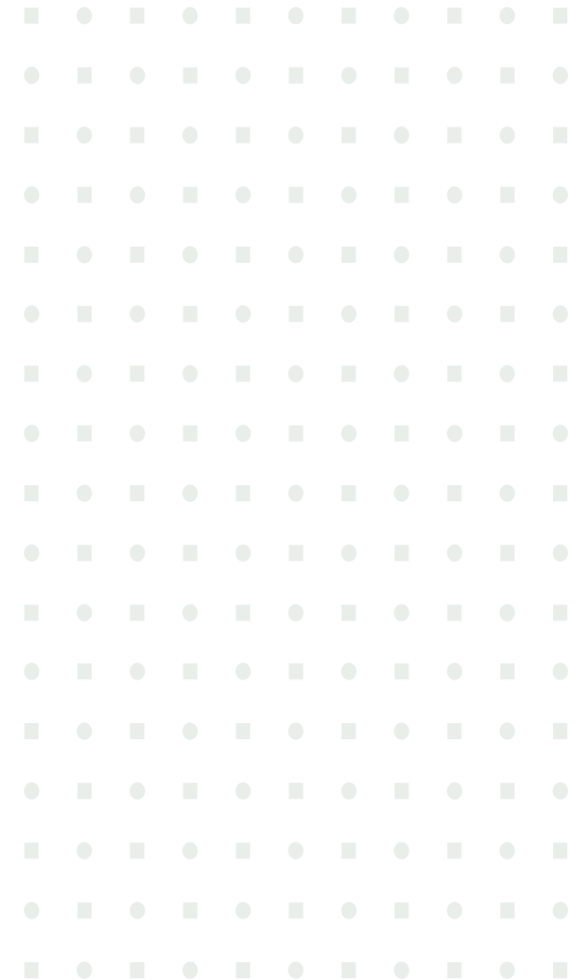
Relationship: within Person

```
...
"RelatedPerson": [
  {
    "ref": "URI TO OTHER PERSON",
    "relationship": {
      "type": [
        "SIS" // Sister
      ],
      "role": [
        "POWATT", // Power of Attorney
        "CAREGIVER" // Carer
      ]
    }
  }
]
```

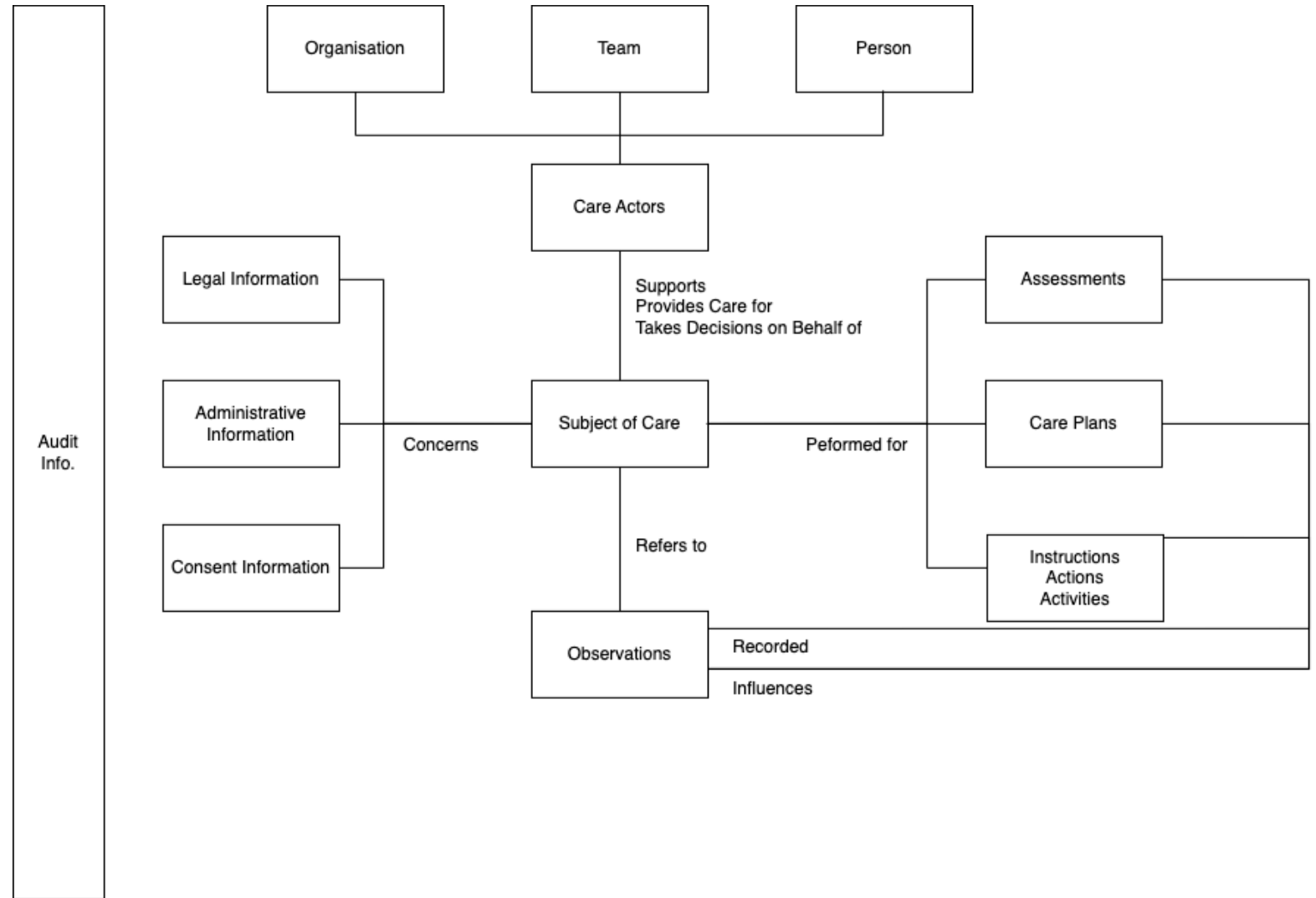
The other person is the sister of the subject, has power of attorney for them, and has primary responsibility for their at-home care

A couple of issues, though

- This does not designate a primary caregiver; a person can have more than one "caregiver"
- Using **ConnectedPerson** necessarily will have very little info about the caregiver, perhaps not even contact details.
 - Do we therefore need a new "Carer" object, with more mandatory information?
- Even using **Person**, are we really capturing enough information?



How does MODS do it? (1)



How does MODS do it? (2)

SubjectOfCare

= our Person spec

UnpaidCarer

=

- Address
- Contact Details
- Name
- Relationship
- In the same household?
- Ordinarily resident of?

How does MODS do it? (2)

SubjectOfCare

= our Person spec

UnpaidCarer

=

- Address
- Contact Details
- Name
- Relationship
- In the same household?
- Ordinarily resident of?

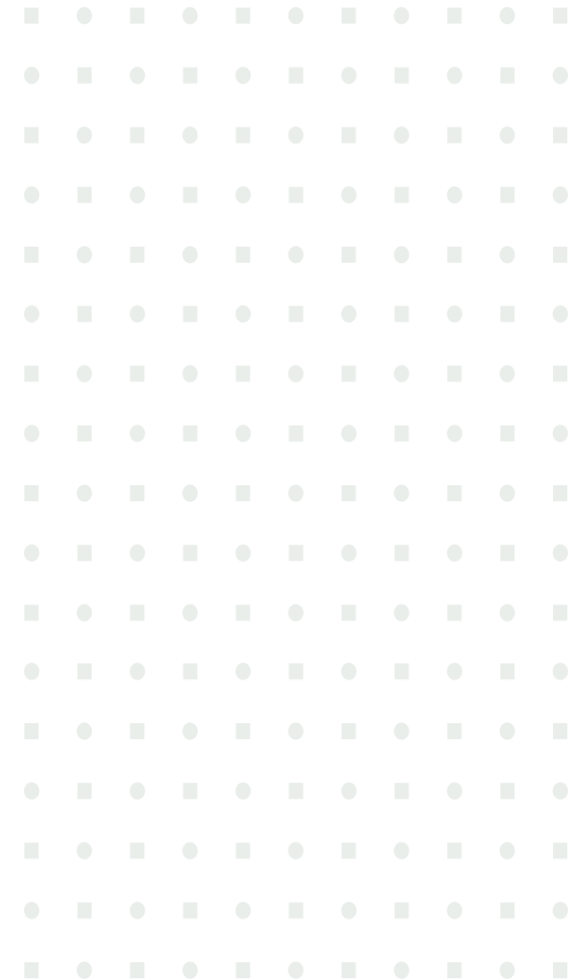
Is that it? {

What needs to be known about unpaid carers?

As we move from "who" to "what", we want to hear from you:

- What information is important to be captured about unpaid carers?
(minimum and ideal)
- Where is that information collected, who by, and who holds it?
(GP survey? Third party?)
- What needs to be standardised?
(information about the carers, information about the support they receive, and information about ...)

No Miro today, so **raise your hand to contribute**. You don't need to answer one of these questions necessarily; just tell us your experiences and stories about carers and the information surrounding them.

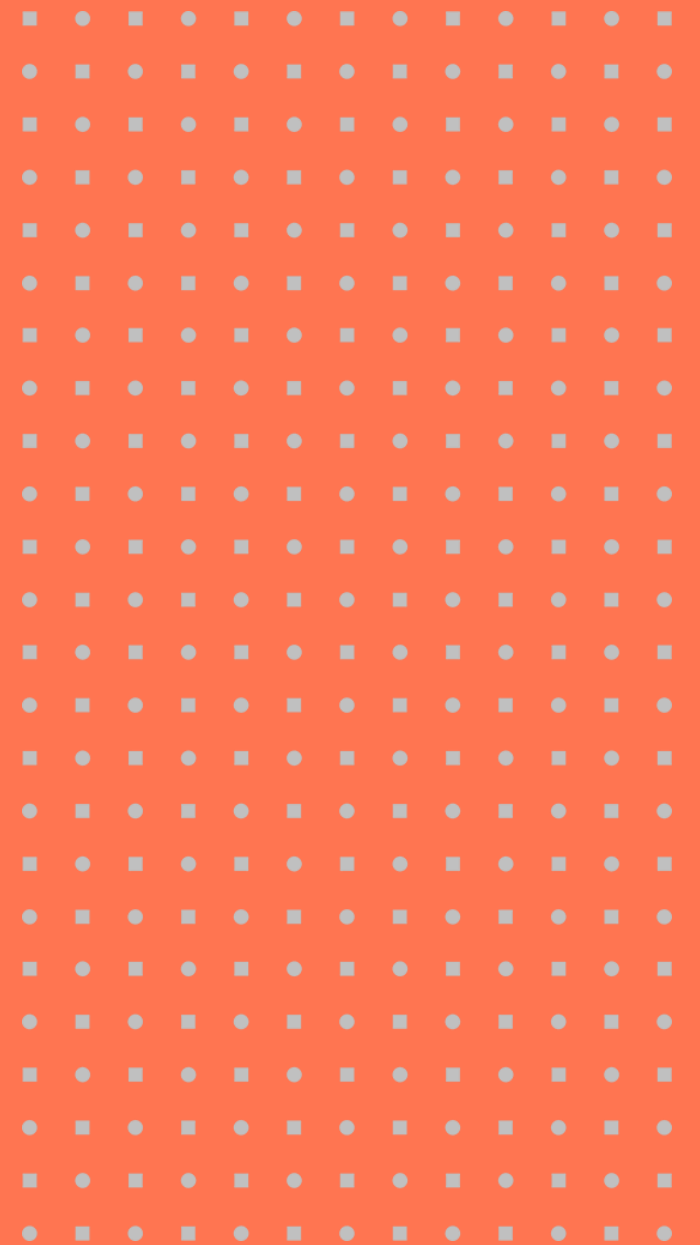


Thanks for your help!



Controlled vocabularies

Demonstration



The data model defines different types of elements

Person standard

Services
standard

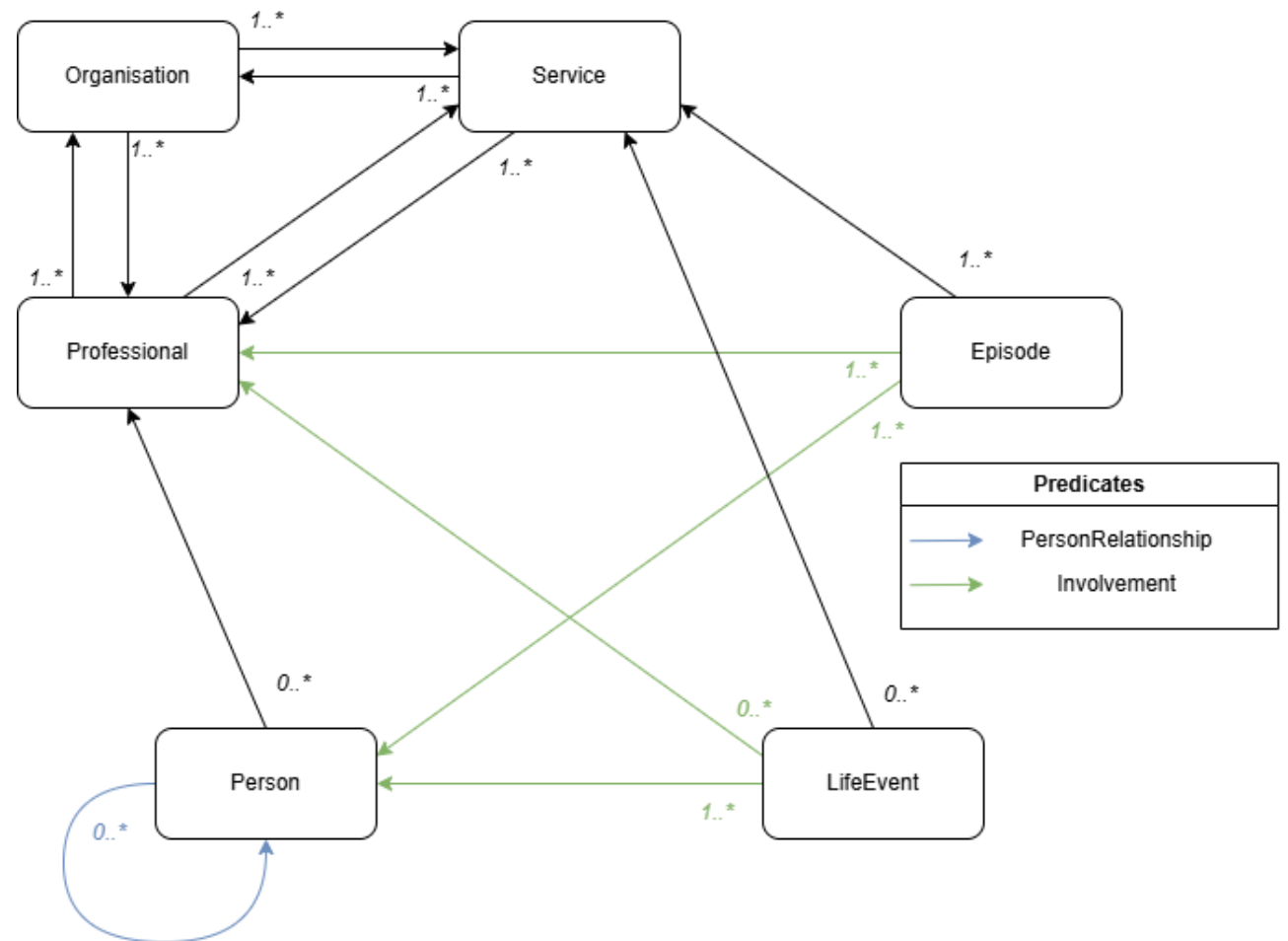
Professional
standard

Relationship
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Organisation
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Life Event standard

Service Episode standard



What kind of

Organisation

Service

Relationship

Professional

Episode

Event

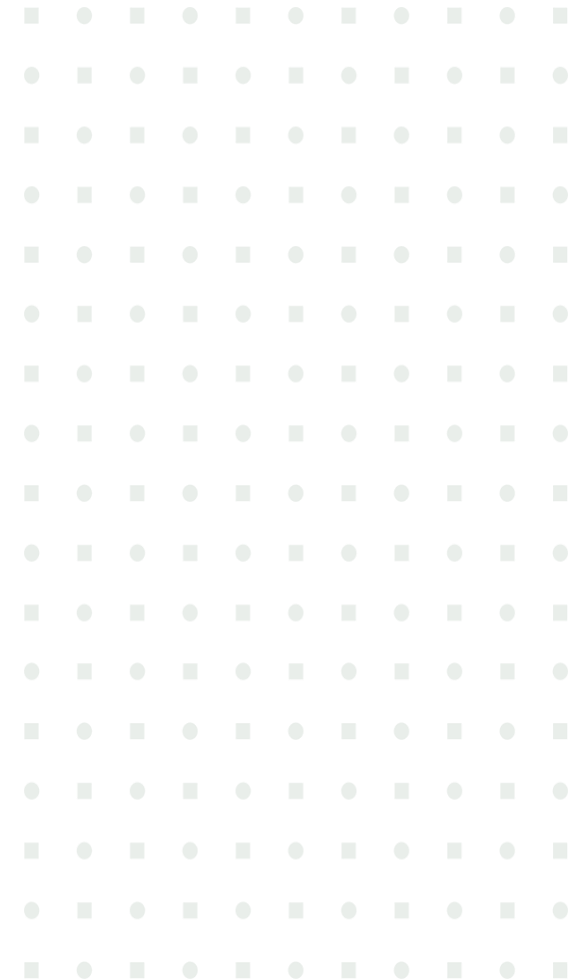
?

Fleshing out the data model with controlled vocabularies

- A controlled vocabulary is a set of terms with singular, agreed-upon definitions, eg.

Display	Definition
Usual	Known as/conventional/the one you normally use.
Official	The formal name as registered in an official (government) registry, but which name might not be commonly used. May be called "legal name".
Temp	A temporary name. Name.period can provide more detailed information. This may also be used for temporary names assigned at birth or in emergency situations.

- It eliminates ambiguity by mapping synonyms to a single preferred term, improving discoverability and understandability
- How do we build a controlled vocab?
 - Use someone else's
 - User research
 - Testing + iteration



What we've done (1)

Item	Controlled vocabulary
Person<>Person Relationship	FHIR <code>PersonalRelationshipRoleType</code>
Person<>Person Role	FHIR <code>relatedperson-relationshiptype</code>
Personal or Professional involvement in events and episodes	FHIR <code>ParticipationType</code>
Person Name Use	FHIR <code>name-use</code>
Person Stated Gender	NHS <code>PERSON STATED GENDER CODE</code>
Person Phenotypic Sex	NHS <code>PERSON PHENOTYPIC SEX</code>
Frequencies	SNOMED <code>Time Patterns</code>

What we've done (2)

Item	Controlled vocabulary
Types of organisation	{Local Authority, NHS Trust, Police} (N=3)
Types of service	{Youth Offending, Primary School, Social Care, ...} (N=5)
Types of episode	{Early Help, EHCP, S47, ...} (N=9)
Types of event	{A&E Attendance, School Suspension, Made Homeless, ...} (N=9)
Types of professional	N/A -- converted to freetext

What we've done (2)

Item	Controlled vocabulary
Types of organisation	{Local Authority, NHS Trust, Police} (N=3)
Types of service	{Youth Offending, Prison, Probation, Social Care, ...} (N=5)
Types of episode	{Early Childhood, ...} (N=9)
Types of event	{Attendance, School Suspension, Made Homeless, ...} (N=9)
Types of professional	N/A -- converted to freetext

EARLY STAGES

What we've done (3)

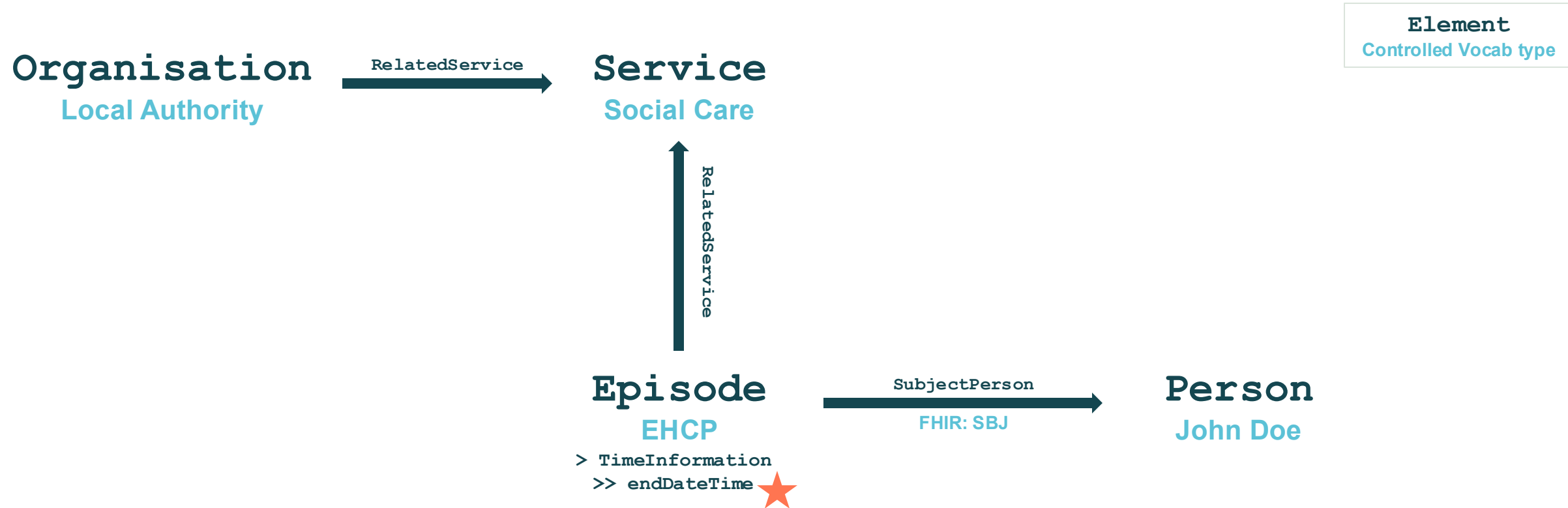
**Session with frontline
practitioners**

**User research from
Hippo Digital**

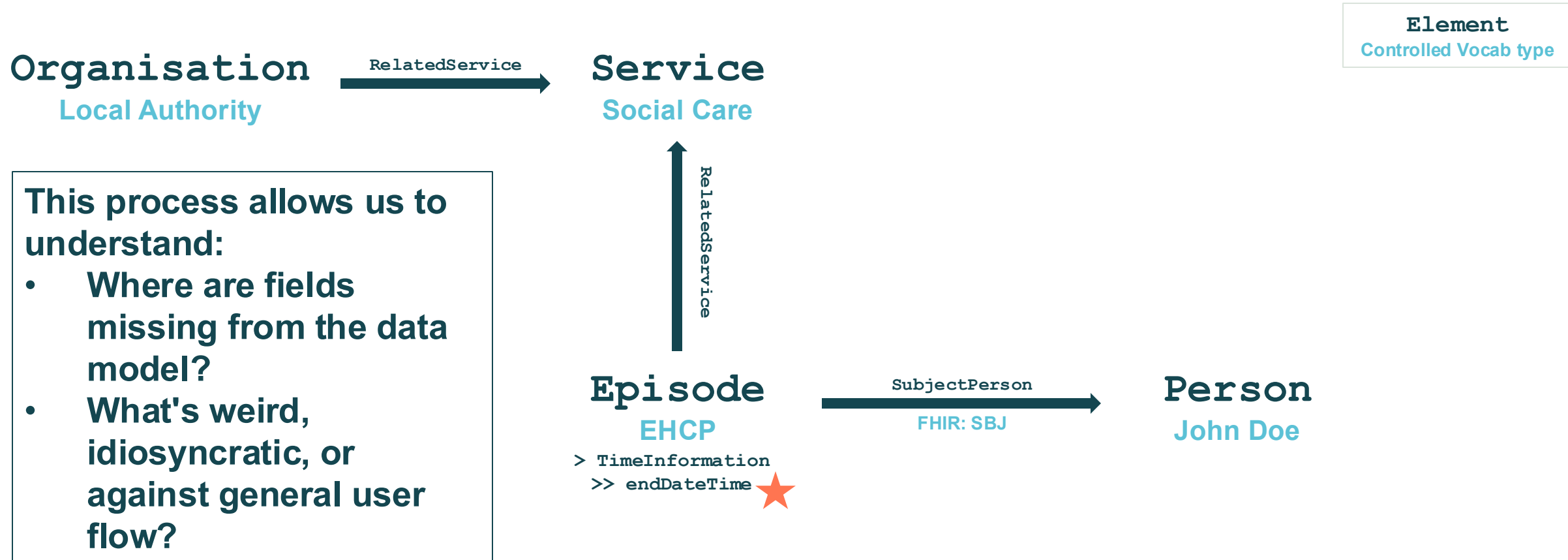
**Current drafts of
controlled vocabularies**

**What user stories can
we currently tell?**

For example: When did John's Education, Health and Care plan begin? (1)



For example: When did John's Education, Health and Care plan begin? (2)



There are loads of items we can't describe yet

Allergies	Do-not-contact	Housing type	Risk of exploitation
Absences from school	Early help referral reasoning	Missed appointments	Risk of gangs and youth violence
Behaviour points	Electively home educated	Missing episode return	Risk of radicalisation
Complaints against	First language	Police markers	Source of referral
Complaints made by	Free School Meals	Police Powers of Protection	Young Justice System offence type
Communication needs	Parental responsibility	Risk level on parent / carer	more...

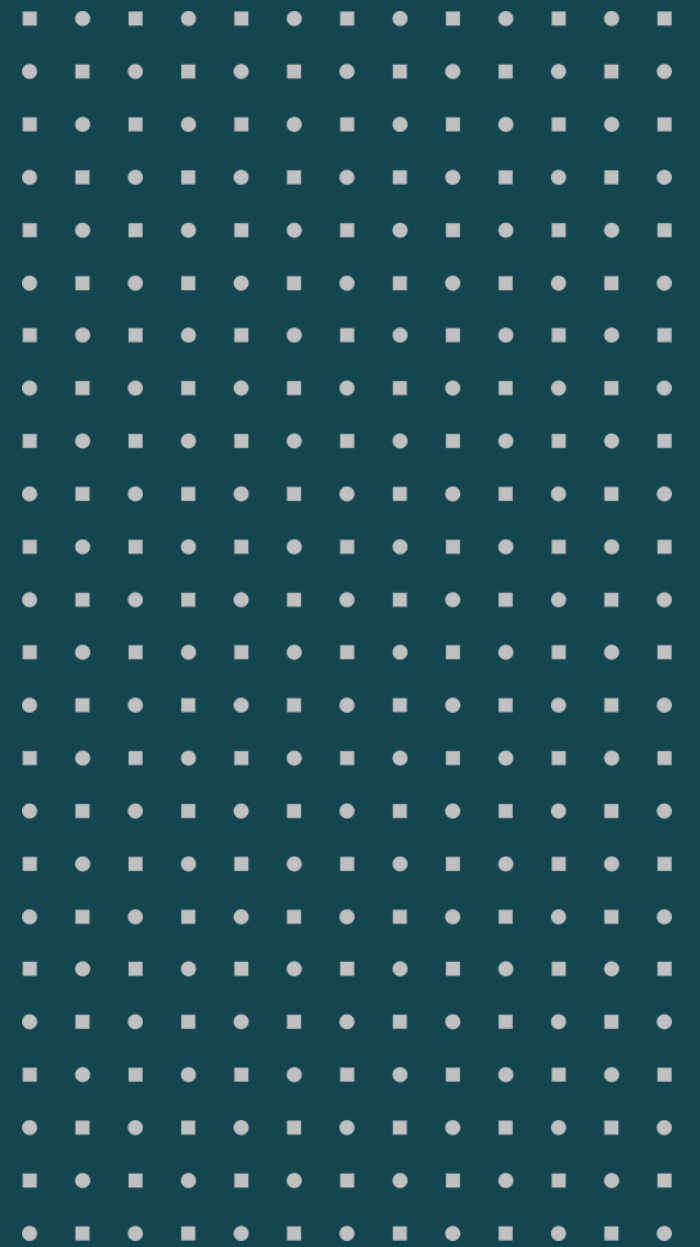


There are loads of items we can't describe yet

Allergies	Do-not-contact	Housing type	Risk of exploitation
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Complaints against	First language	Police markers	Source of referral
Complaints made by	Free School Meals	Police Power Protection	
Communication needs	Parental responsibility	Risk level of carer	

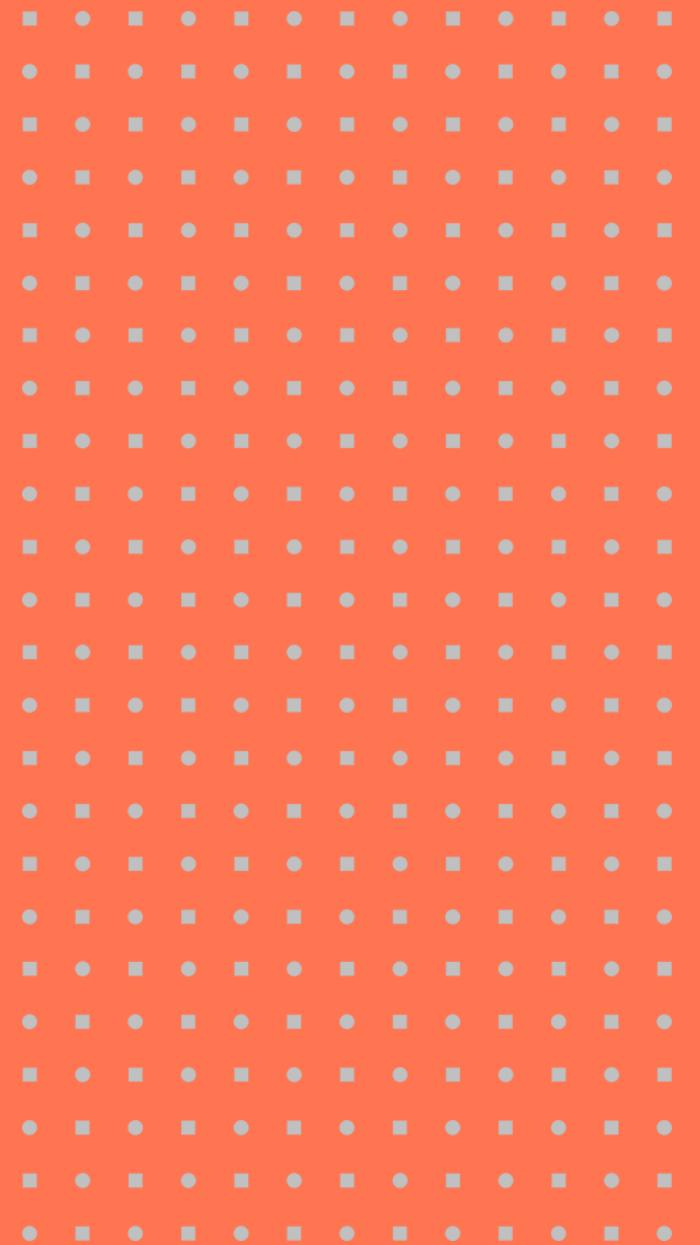
If you know any controlled vocabs that we could adopt or adapt, please let us know

Stay tuned!



Assessments and Plans

Interactive session



Why assessments and plans?

Assessments and **plans** contain important information about a person and their care that needs to be shared between people and professionals in order to enable **joined up care**.

Context

- Currently we don't think this information can be described consistently as LAs have the autonomy to collect the data in different ways
 - *We don't want to directly influence practice or LA processes*
- However, we want to consider making the architecture more consistent so that we can have **comparable outputs that can be shared**

How should we do this?

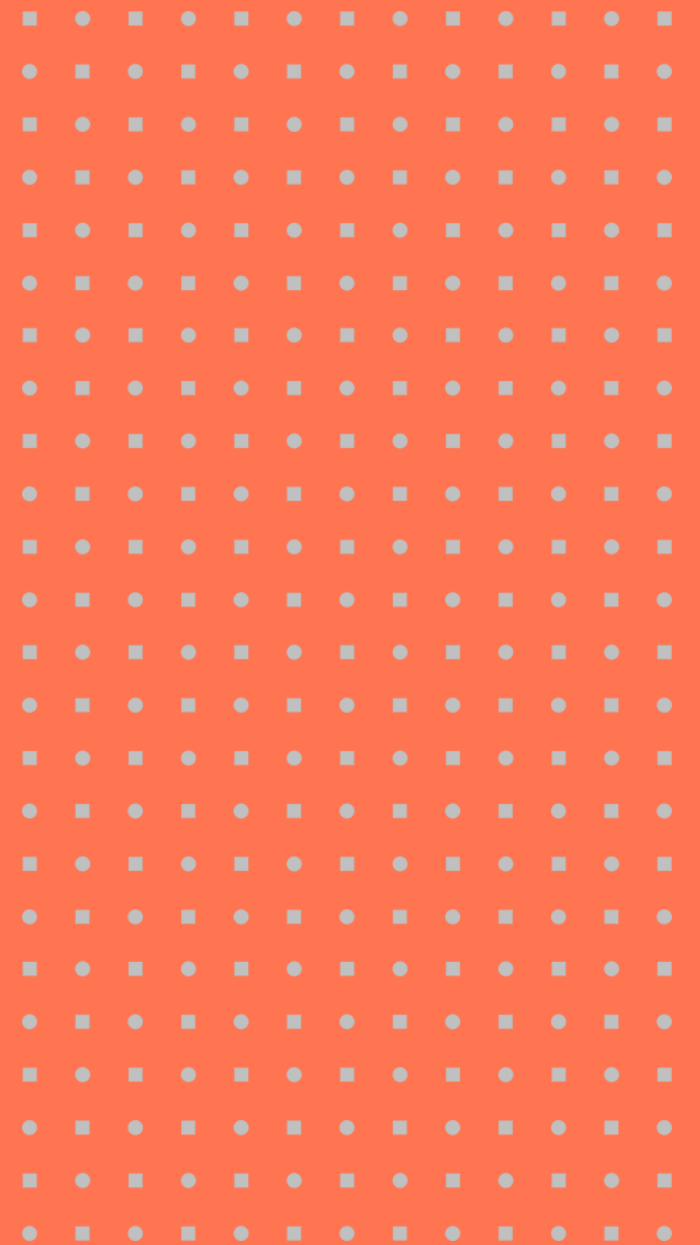
Metadata only: i.e. plan
created by X on Y date

Standardise key information i.e.
outcome of an assessment

Standardise categories of information e.g. needs,
strengths

Standardise sub-categories and fields

Wrapping up + Next steps



Next steps

- Identify the best way forward regarding Unpaid Carers
- Iterate and test controlled vocabularies and our overarching data model.
- Continue research and propose approach to assessments and plans.
- Iterate proposals for the interoperability framework.
- Finalise next version of our website – to be published soon.

Homework: Review and raise issues on the Controlled Vocabularies:

- <https://socialcaredata.github.io/spec/controlled-vocabularies/?tab=specification>
- <https://github.com/SocialCareData/controlled-vocabularies/issues>
- Consider:
 - What's missing? Vocabs and Terms
 - Are any definitions problematic?
 - Other sources we should investigate?

- **Next Working Group Meeting: 28th May**

Thank you.

Contact:

Andrew.newman@theodi.org

